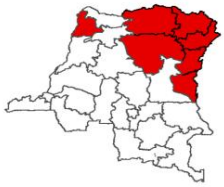
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Ministry of Public Health, Hygiene and Social Welfare
National Institute of Public Health
Public Health Emergency Operations Center
PRESIDENTIAL EBOLA TASK FORCE 17
MVE17 Incident Management System



Situation Report of the 17th Ebola Virus Disease Outbreak / DRC

SitRep N°163/MVE-BDBV/24/10/2026

Reporting date: October 24, 2026 Publication date: October 25, 2026

HIGHLIGHTS (last 24 hours)

Eighty-eight (88) new confirmed cases and 38 deaths (26 community deaths and 12 in Ebola Treatment Centers) of Ebola Virus Disease (EVD) were reported in the provinces of Ituri (44), North Kivu (39) and Haut-Uélé (5), for a total of 512 new samples were analyzed; 88 of these samples were found to be positive for the Bundibugyo virus, representing a positivity rate of 17.2%.

No new health zones have been affected in the last 24 hours.

ACCUMULATIVE CASE	ACCUMULATIVE CONFIRMED DEATHS	LETHALITY	PATIENTS IN ISOLATION/CTE	ACCUMULATIVE HEALED	TRACKING RATES CONTACTS (Of the Day)
9,964	4841	48.6%	1008	2,670	87.1%

Affected provinces and affected health zones

8 Provinces affected

Haut-Uélé, Ituri, North Kivu, South Kivu, Tshopo, Bas-Uélé, South Ubangi, and North Ubangi

66 Health Zones affected out of 178 (37.1%)

Summary of key points from the last 24 hours

As of October 24, 2026, **88 new confirmed cases** of Ebola virus disease (EVD) were reported in three provinces : **Ituri (44 cases), North Kivu (39 cases), and Haut-Uélé (5 cases).** Among these cases, **26 community deaths** were reported **(14 in North Kivu, 11 in Ituri, and 1 in Haut-Uélé).** In addition, **12 confirmed deaths were recorded in Ebola Treatment Centers (6 in Ituri and 6 in North Kivu),** bringing **the total number of confirmed deaths reported** during this period to 38.

Furthermore, 24 **patients (12 in North Kivu, 9 in Ituri and 3 in Bas-Uélé)** were declared recovered after obtaining two negative results in the Ebola virus disease (EVD) follow-up test.

o As of October 24, 2026, the DRC has accumulated **9,964 confirmed cases, including 4,841** deaths, representing a case fatality rate **of 48.6%.** Ituri province remains the epicenter, accounting for **72.0** % of reported confirmed cases and **50.0%** new confirmed cases, while North Kivu (44.3% of new cases) shows a lethality of 56.1%, higher than the national average.

No new health zones have been affected in the last few months.

24 hours. Thus, 66 out of 178 health zones (37.1%), spread across eight provinces, have been affected since the start of the epidemic.

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Table 1. Distribution of confirmed cases and deaths by affected province, as of October 24, 2026.

Province	New confirmed cases (24h)	Confirmed cases	Deaths (confirmed)	Lethality	Affected health zones
Ituri	44	7179	3,355	46.7%	28/36 (77.8%)
North Kivu	39	2251	1,262	56.1%	17/34 (50.0%)
Haut-Uélé	5	436	190	43.6%	7/13 (53.8%)
Tshopo	0	77	27	35.1%	7/23 (30.4%)
South Kivu	0	3	1	33.3%	1/34 (2.9%)
Bas-Uélé	0	13	4	30.8%	4/11 (36.4%)
South Ubangi	0	2	1	50.0%	1/16 (6.3%)
North Ubangi	0	3	1	33.3%	1/11 (9.1%)
Total	**88**	**9,964**	**4,841**	**48.6%**	**66/17 8(37.1%)**

Table 2. Distribution of confirmed cases and deaths by province and zone as of October 24, 2026

Province / Health Zone	Cumulative number			Current situation (24 hours)			
	Case (n)	Deaths (n)	Lethality	New Case	Community deaths (Swab+)	Confirmed deaths within the CTE	Total confirmed deaths
Ituri	**7179**	**3355**	**46.7%**	**44**	**11**	**6**	**17**
Adja	11	1	9.1%				0
Aru	23	6	26.1%	3	0	0	0
Ariwara	8	2	25.0%				0
Aungba	12	5	41.7%				0
Bamboo	218	48	22.0%	2	0	0	0
Boga	2	2	100.0%				0
Bunia	1941	661	34.1%	6	2	3	5
Damascus	41	10	24.4%				0
Drodro	15	7	46.7%				0
Fataki	84	31	36.9%				0
Gethy	11	7	63.6%				0
Kambala	2	0	0.0%				0
Kilo	38	12	31.6%				0
Komanda	192	118	61.5%				0
Lita	319	161	50.5%	2	2		2
Logo	14	9	64.3%				0
Lolwa	36	9	25.0%				0
Mahagi	5	1	20.0%				0
Mambasa	81	37	45.7%	2	2	0	2
Mandima	422	227	53.8%	20	4	0	4
Mangala	386	168	43.5%	2	0	0	0

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Mongbwalu	717	346	48.3%				0
Neither a -Nia	338	197	58.3%				0
Nizi	839	306	36.5%	2	0	0	0
Nyankunde	135	42	31.1%	1	0	1	1
Rimba	21	5	23.8%				0
Rwampara	1183	417	35.2%	4	1	2	3
Tchomia	85	27	31.8%				0
To ventilate	N / A	493	N / A				0
North Kivu	**2251**	**1262**	**56.1%**	**39**	**14**	**6**	**20**
Blessed	586	397	67.7%	16	11	0	11
Good	24	8	33.3%				0
Butembo	328	191	58.2%	2	0	1	1
Goma	1	0	0.0%				0
Kalunguta	90	36	40.0%	3	1	2	3
Katwa	659	402	61.0%	3	0	1	1
Kayna	3	2	66.7%				0
Kyondo	45	21	46.7%	1	0	0	0
Lubero	37	14	37.8%				0
Mabalako	98	36	36.7%	5	1		1
Alimbongo	5	3	60.0%				0
Manguredjipa	16	5	31.3%				0
Masereka	39	13	33.3%				0
Musienene	156	74	47.4%			1	1
Mutwanga	42	12	28.6%	5	0	0	0
Oicha	66	30	45.5%	3	0	0	0
Vuhovi	56	18	32.1%	1	1	1	2
Upper Uele	**436**	**190**	**43.6%**	**5**	**1**	**0**	**1**
Boma Mangbetu	45	19	42.2%				0
Dungu	1	1	100.0%				0
Gombari	3	1	33.3%				0
Isiro	109	51	46.8%				0
Pawa	109	62	56.9%	2	1		1
Rungu	6	3	50.0%				0
Wamba	163	53	32.5%	3	0		0
Tshopo	**77**	**27**	**35.1%**	0	0	0	0
Bafwasende	18	3	16.7%				0
Kabondo	11	2	18.2%				0
Lubunga	1	0	0.0%				0
Makiso - Kisangani	33	14	42.4%				0
Mangobo	10	7	70.0%				0
Tshopo	3	0	0.0%				0

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Wanie-Rukula	1	1	100.0%				0
South Kivu	**3**	**1**	**33.3%**	0	0	0	0
Miti-Murhesa	3	1	33.3%				0
Bas Uélé	**13**	**4**	**30.8%**	0	0	0	0
Buta	1	1	100.0%				0
Viadana	6	1	16.7%				0
Dakwa	1	0	0.0%				0
Ganga	5	2	40.0%				0
South Ubangi	**2**	**1**	**50.0%**	0	0	0	0
Bulu	2	1	50.0%				0
North Ubangi	**3**	**1**	**33.3%**	0	0	0	0
Wapinda	3	1	33.3%				
Total	**9964**	**4841**	**48.6%**	**88**	**26**	**12**	**38**

***These are the confirmed deaths that occurred in the various CTEs in the province which are being broken down to be classified by health zone.**

NA: Not Applicable.

Table 3. Status of alerts notified by province for October 24, 2026

DPS	Alerts Received		Total alerts	Alerts verified and validated (suspected case)		Alerts verified and invalidated (no suspected cases)		Number of suspects investigated	Number of suspects investigated and transferred to CT/CTE (of the day)
	Living	Deceased		Living	Deceased	Living	Deceased		
Haut-Uélé	131	4	135	18	1	113	3	19	5
Bas Uélé	ND	ND	ND	ND	ND	ND	ND	ND	ND
Ituri	1,273	61	1,334	122	61	1,151	0	168	102
North Kivu	1,492	57	1,549	160	57	1,332	0	183	126
South Ubangi	ND	ND	ND	ND	ND	ND	ND	ND	ND
North Ubangi	5	0	5	5	0	0	0	5	4
Tshopo	ND	ND	ND	ND	ND	ND	ND	ND	ND
Grand Total	**2,901**	**122**	**3,023**	**305**	**119**	**2,596**	**3**	**375**	**237**

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Figure 1. Evolution of cases by date of symptom onset over the last 21 days

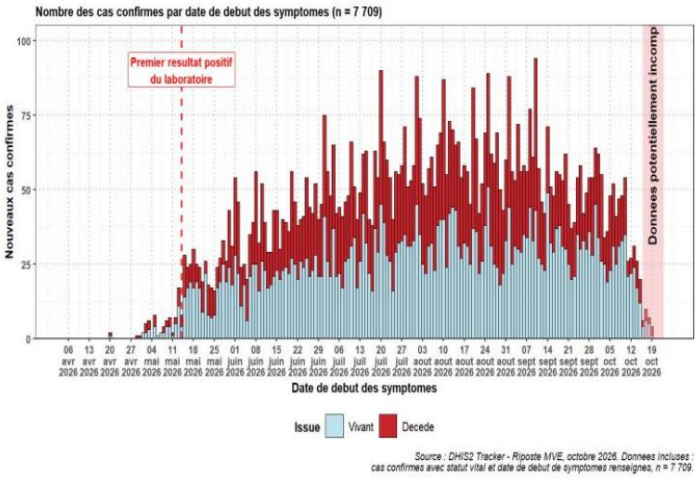
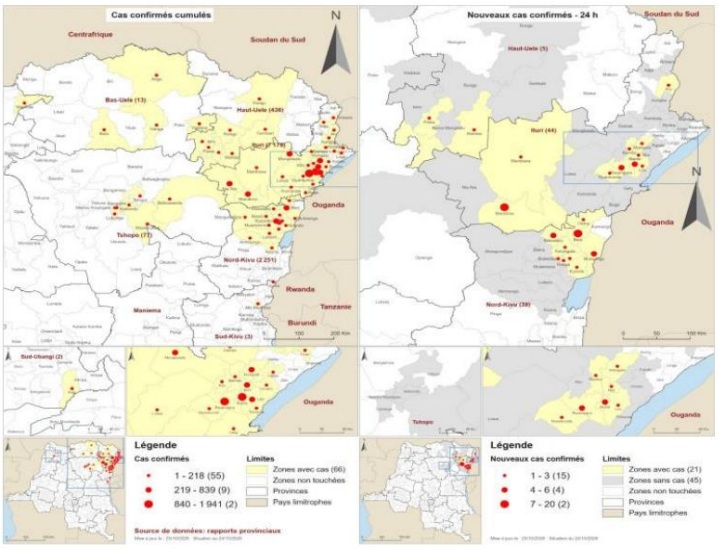


Figure 2. Spatial distribution of confirmed cases (24 hours and cumulative) by health zone.



1. COORDINATION

1.1. Main Actions

ÿ Support for response activities in the affected provinces through the technical pillars.

2. RESPONSE ACTIONS BY PILLAR

2.1. Epidemiological surveillance

2.1.1. Main Actions

ÿ As of October 24, 2026, a total of 3,023 alerts were reported in the four provinces that submitted their data, of which 424 were validated as suspected cases (14.0%). Of these suspects, 375 were investigated (88.4%).

ÿ Regarding contact tracing, 32,697 out of 37,552 contacts were seen in the last 24 hours, representing a follow-up rate of 87.1%, down from the previous day (88.4%) and below the expected threshold of 95%. North Kivu has the lowest follow-up rate (82.4%), followed by Haut-Uélé (88.9%), Ituri (92.2%), and Bas-Uélé (99.5%).

2.1.2. Update on checkpoint and point of entry (PoE/PoC) activities

2.1.3. Main actions

ÿ On October 24, 2026, eighteen (18) alerts were notified to the PoE/PoC: 4 in Ituri, 1 in Haut-Uélé, and 13 in Bas-Uélé. In Ituri, one body was intercepted approximately 400 meters from the Poipo PoC (Mongbwalu), validated, and swabé; the live alert from the Marabo PoC (Nyankunde) was validated and transferred to the Nyankunde CTE.

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Table 4. Status of key PoE/PoC indicators by province, as of October 24, 2026

Indicators	Ituri	North Kivu	South Kivu	Tshopo	Haut-Uélé	Bas-Uélé	Overall
Completeness of the reports	98.4% (61/62)	95% (20/21)	56% (5/9)	100% (18/18)	100% (10/10)	85% (17/20)	94% (131/140)
Strategic PoC/PoE	1	0	0	0	0	0	1
Number of past travelers	161,879	93,284	7,435	18,961	25,160	6,857	313,576
Proportion of the travelers screened	97%	98%	98%	99%	99%	98%	98%
% of travelers who washed their hands	97%	98%	98%	99%	99%	99%	98%
% of travelers made aware	99%	98%	100%	100%	100%	91%	99%
Number alerts notified	04	0	0	0	01	13	18
Number alerts validated live	02	0	0	0	01	02	5
Number of lifeless bodies intercepted	01	0	0	0	0	0	1
% of suspected cases transferred to CT/CTE	50% (1/2)	0%	0%	0%	100%	100%	80% (4/5)
% of lifeless bodies swabed/ secured	100%	0%	0%	0%	0%	0%	100% (1/1)
% of suspected cases investigated within 24 hours	100% (3/3)	0%	0%	0%	100%	100%	100% (6/6)

2.1.4. Challenges

ÿ In Ituri, the Avakubi PoC remains non-functional due to insecurity, the alert validated at the Bela PoC could not be transferred due to lack of ambulance (50%; 1/2) and 39% of travelers refused screening at the Nzebi PoC (921/2 354); in South Kivu, the completeness of reports remains low (56%; 5/9) and the Kavimvira, Kalundu Port SNCC and Baraka PoEs do not have isolation facilities; in Bas-Uélé, 3 out of 20 PoCs did not report (Bale, Mangite and Garre Dingila; 85%; 17/20), only 91% of travelers were sensitized and the PNHF services remain non-functional.

2.2. Laboratory

2.2.1. Main actions

- **Ituri: 44 new positive results** (33 living and 11 deaths) out of 257 new samples analyzed (positivity rate: 17.1%)

ÿ **North Kivu: 39 new positive results** (25 living and 14 dead) out of 229 new samples analyzed in Beni and Butembo (positivity: 17.0%).

ÿ **Haut-Uélé: 5 new positive results** (4 living and 1 death) out of 18 new samples analyzed (positivity: 27.8%).

ÿ **Bas Uélé: 8 samples received and analyzed (8 live) the results were negative.**

ÿ 12 samples are currently being shipped to the Gbadolite laboratory (10 of which are from Wapinda, 1 Yakoma and 1 in Bosobolo),

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2.2.2. Infection Prevention and Control (IPC/EDS)

2.2.3. Main actions

ÿ **Ituri :** 35 out of 38 rings were opened (92.1%), 44 out of 50 households were decontaminated (88.0%), and 10 out of 10 health facilities were decontaminated (100.0%). Of 78 EDS alerts (including 8 postponements), 56 EDS were carried out (71.8%) and 56 bodies were swabed.

ÿ **North Kivu :** 14 out of 22 rings were opened (63.6%), 11 out of 13 health centers were decontaminated (84.6%), and 8 out of 17 households (47.1%) were decontaminated. None of the 8 decontaminated households received supplies. 8 infection prevention and control (IPC)/ water, sanitation, and hygiene (WASH) assessments were conducted in 8 health centers (average score of 28.4%), 150 health center follow-ups were carried out, 277 service providers were briefed, 3 health centers received IPC kits, and 23 schools received hand sanitizer (20 of which were in Musienene). Of 61 emergency health alerts, 53 were carried out and 61 bodies were swabbed.

ÿ **Haut-Uélé :** 2 out of 2 rings were opened (100.0%) in Wamba, 31 out of 31 households were decontaminated (100.0%) (21 in Boma Mangbetu, 9 in Wamba, and 1 in Isiro), and 3 out of 3 ESS (100.0%) in Isiro, Boma Mangbetu, and Wamba; 23 service providers were briefed (9 in Wamba, 7 in Isiro, and 7 in Pawa), the Etakamukongo Health Center was assessed in Pawa (52%), and 1 triage unit is operational at the Tshakatshaka Health Center (Wamba). Of 4 EDS alerts, 4 EDS were carried out (100.0%), and 4 bodies were swabed.

2.2.4. Challenges.

In Ituri, 22 out of 78 health assessments (28.2%) could not be carried out, and resistance was recorded in Bambu, Mangala, Fataki, and Mandima. In North Kivu, the opening of the treatment rings remains insufficient (63.6%; 14/22), with gaps in Katwa, Beni, and Oicha; less than half of the identified households were decontaminated (47.1%; 8/17); no decontaminated household received supplies (0/8); the average IPC/WASH score of the assessed health establishments remains low (28.4%); and 50 health assessments were postponed to the following day due to a lack of agreement from families. In Haut-Uélé, only 4 out of 7 health zones reported, IPC kits are lacking in the health establishments, and IPC supervisors are not activated in 4 health zones (Rungu, Dungu, Boma Mangbetu, and Gombari).

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2.2.5. Holistic care

2.2.6. Main actions

ÿ **In Ituri,** 88 new admissions were recorded compared to 76 discharges, including 9 recoveries. Thus, 422 patients are hospitalized, representing an occupancy rate of 40%, 391 out of 406 patients (96.3%) received hot meals and all children under LNPE/SLM were taken care of (16/16 at the CTE, 5/5 at the nursery and 54/54 in the community).

ÿ **In North Kivu,** 156 new admissions (16 confirmed and 140 suspected) were recorded, compared to 108 discharges, including 12 recovered patients. Thus, 495 patients are currently receiving care, including 68 children under 5 years old, 20 critically ill patients, and 147 patients outside of Ebola Treatment Centers (ETCs), for an overall occupancy rate of 84.9% (348/410) for ETCs; 958 beneficiaries received hot meals, and 28 ambulance trips were successfully completed (12 out of 14 ambulances used).

ÿ **In Haut-Uélé,** 15 new admissions were recorded against 6 discharges; 41 patients are hospitalized for 158 beds, representing an occupancy rate of 25.9%, and all benefited from hot meals, as did 68 accompanying persons and 91 PPLs.

In Bas **-Uélé,** 15 new admissions were recorded, compared to 20 discharges, including 17 non-cases and 3 recoveries. Thus, 92 patients remain in isolation at the end of the day. In Nord-Ubangi, one confirmed case escaped from isolation at Wapinda General Referral Hospital, care continues for the other confirmed case, and one suspected case has been isolated in Bosobolo.

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2.2.7. Challenges

ÿ **In North Kivu,** 147 patients (2 confirmed and 145 suspected) remain under care outside CTE/CT, beds for suspected cases are almost saturated (97.9%; 188/192) and ambulance response capacity remains limited (50%; 14/28).

ÿ **In North Ubangi,** the second confirmed case escaped from isolation at the Wapinda General Referral Hospital, the province has neither a CTE nor an isolation framework for suspects and the family of a suspect from Yakoma refuses his isolation.

ÿ **In Haut-Uélé,** the data on care have not been reported and a suspected case of 16 months has died at the CTE in Pawa; in Ituri, blood remains insufficient in relation to the needs of the CTEs and hospitals, 15 patients in Mandima have not received food, the capacity to receive patients remains low in some CTE/CTs (Aru, Mambasa, Kilo, Lolwa, Mandima and Biakato) and the reporting of ambulance movements remains.

2.2.8. Vaccination 2.2.9.

Main actions

In Ituri, vaccination of people with low blood protein (LBP) using Ervebo and research on its efficacy against BDBV continue, bringing the cumulative total to 1,613 people vaccinated (963 men and 650 women, including 21 pregnant women and 39 breastfeeding women), with 31 reported adverse events following immunization (AEFIs) (26 non-severe and 5 severe). Clinical trials for the TUMAINI project have been officially launched. In Bas-Uélé, the cumulative total has reached 1,660 people vaccinated out of a target population of 13,202 (12.6%), primarily in Buta (648; 22.6%) and Ganga (273; 20.5%). Compassionate vaccination continues in the health zones of Ango and Buta, and pre-registration of LBP and TBP is underway. In Haut-Uélé, LBP vaccination remains suspended.

Data from North Kivu and Tshopo has not yet arrived.

2.2.10. Challenges

ÿ In Bas-Uélé, the start-up of the UCCs is not effective and vaccines as well as vaccination funds remain insufficient; in Haut-Uélé, the breakdown of the generator at the PEV antenna in Isiro prevents the start-up of the 2 UCCs and the vaccination of PPLs; in Ituri, the target of TPLs and PPLs remains to be redefined and 5 serious AFIs have been notified.

2.2.11. Continuity of Care

2.2.12. Main actions ÿ In Tshopo, the

Bombula, Pharmat and St Francis of Assisi health centers were evaluated: the triage area is not effective in any of the visited health centers, the partogram is not used for monitoring deliveries (Bombula and Pharmat), 12 children missed their CPS appointment (8 at Pharmat and 4 at Bombula) and 14 out of 16 staff (87.5%) are vaccinated at St Francis of Assisi; the recovery plan for the Marie Antoinette health center was developed.

2.2.13. Challenges

ÿ North Kivu, CSSE plan and absenteeism contingency plan to be developed at Katwa General Hospital. In Ituri, only 472 out of 717 recovered patients (65.8%) are being monitored, the Bunia recovery clinic is overused and some medications and supplies are out of stock; implementation of recovery plans for assessed social and solidarity economy (SSE) facilities still to be ensured and incinerators not operational (St. Francis of Assisi and Pharmat).

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2.3. Risk Communication and Community Engagement (RCCE)

2.3.1. Main actions

ÿ In Ituri, 23,793 home visits reached 76,422 people, 747 educational talks reached 11,496 people, 82 mass awareness campaigns reached 3,659 people and 1 community debate reached 43 people; 406 community alerts (including 213 deaths) were notified, 39 out of 48 feedbacks (81.3%) and 6 out of 17 incidents (35.3%) were resolved within 48 hours, the pre-listing of 210 contacts around 12 confirmed cases was facilitated and 14 awareness spots were broadcast in 36 radio stations; only 10 out of 28 health zones (35.7%) reported.

ÿ In North Kivu, 16,374 home visits reached 51,518 people, 294 educational talks reached 6,502 people, 46 mass communications reached 9,024 people and 6 briefings reached 179 people; 445 out of 650 community alerts (68.5%) were notified, 23 out of 31 feedbacks (74.2%) and 1 out of 8 incidents (12.5%) were resolved within 48 hours, and 2,186 out of 4,750 people (46.0%) engaged in community actions; advocacy was carried out with the Director of the Educational Province (Katwa), a community dialogue brought together hairdressers in Musienene and 54 community actors were briefed in Mutwanga.

ÿ In Haut-Uélé, 45 home visits reached 362 people, 25 educational talks reached 592 people and 3 mass awareness sessions in schools reached 3,542 people in Isiro; 5 community alerts were notified in Wamba and 5 out of 5 feedbacks were resolved within 48 hours; the Vatican village committee was set up and 10 motorcyclists were trained in Isiro, while the activities of the Bole Bole transit center were suspended following an incident that occurred after a death.

2.3.2. Mental Health and Psychosocial Support (MHPSS)

2.3.3. Main actions

ÿ In Ituri, 22 out of 28 confirmed cases (78.6%) and 70 confirmed cases under follow-up out of 125 benefited from SMSPS interventions in 15 affected health zones covered out of 28 (53.6%), 43 new suspects out of 87 and 128 suspects under follow-up out of 189 were taken care of, 14 out of 16 separated and orphaned child households (87.5%) were accompanied, 64 children are being followed up, 6 out of 6 children are in daycare, 3 family reunifications were carried out, 9 recovered patients were reintegrated and 36 out of 37 recovered patients are being followed up in the community; 97 results were announced, including 22 positive ones, and 244 accompanying persons and 65 PPLs were supported.

ÿ In North Kivu, 21 out of 28 confirmed cases (75.0%) and 196 confirmed cases under follow-up benefited from SMSPS interventions in 7 out of 7 health zones (100%), 238 new suspects and 245 suspects under follow-up were taken care of, 37 out of 40 (92.5%) households of separated and orphaned children were accompanied, 28 out of 36 children (77.8%) were followed up in the community, 6 out of 6 children are in daycare, 8 recovered patients were reintegrated and 10 out of 12 (83.3%) recovered patients are followed up in the community; 108 results were announced, including 28 positive results, 517 people (183 men, 262 women and 72 children) benefited from psychoeducation sessions, 89 households were accompanied and 65 PPLs were supported.

ÿ In Haut-Uélé, 2 confirmed cases under follow-up, 2 new suspects and 8 suspects under follow-up were taken care of in Isiro and Pawa, 12 results were announced, all negative, 4 separated children are being followed in the community, 18 escorts and 30 PPLs were supported and 1 EDS supported; data from Boma Mangbetu and Wamba were not received.

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2.3.4. Challenges

ÿ In Ituri, the coverage of interventions around cases remains insufficient in Bunia (38%; 3/8); in North Kivu, SMSPS coverage remains low around the latest confirmed cases (75.0%; 21/28), particularly in Biena, Manguredjipa, Kayna, Lubero, Masereka, Kalunguta, Mabalako, Mutwanga and Vuhovi, 10 affected health zones do not have a nursery and human resources trained in SMSPS remain insufficient; in Haut-Uélé, no nursery is available for separated children (Isiro, Pawa, Wamba and Boma Mangbetu), the mobility of APS is lacking in Wamba and Boma Mangbetu, and nutritional kits for weaned infants as well as the care of orphans remain insufficient.

2.3.5. Logistics
2.3.6. Main actions

ÿ In North Kivu, IPC, PPE and EDS inputs were delivered to the INRB laboratory (Tyvek, nitrile gloves, single-use gowns, gel, garbage bags and chlorine), to the Mabalako Health Zone (body bags for children and adults) and to the Beni Health Zone (thermoflash), and inputs received from WHO from Bunia were unpacked; in North Ubangi, the deployment of IPC inputs and PEC medicines continues in Yakoma and Wapinda and the Starlink resubscription of the 11 Health Zones is being monitored with WHO.

2.3.7. Security

2.3.8. Main actions

ÿ In North Kivu, EDS teams were accompanied for the burial of 3 confirmed cases at the Bundji public cemetery, and IPC teams for the decontamination of a household in AS Mandrandele and the Kitumaini school in AS Butanuka; the Butuhe PoC remains closed since the incident of 26/09/2026 and, in Ituri, the Avakubi PoC remains non-functional due to insecurity.

2.3.9. PSEA (Prevention of Sexual Exploitation and Abuse)
2.3.10. Main actions

ÿ In North Kivu, 17 community health workers (12 women and 5 men) and 76 people (38 men and 38 women) were sensitized on the PSEAH (Protection of School-Based Child Health) and reporting procedures in the Muzebere health district (Katwa), as well as 53 participants in the Mukulya health district (Beni) and 23 women who gave birth at the Kalunguta General Referral Hospital. In Ituri, training on safeguarding, PSEAH, and risk analysis in schools was completed in Gethy for 24 participants (14 men and 10 women) with the support of Save the Children, and the display of messages continues in 6 health zones. In Haut-Uélé, six LLIN distribution agents from the BCZS in Isiro and 12 service providers from the CECCA 16 PoC in Wamba (7 men and 5 women) received training on the PSEA (Protection of Animal Health), referral mechanisms, and the reporting process. Nationally, 2,381 people were sensitized and 59 service providers briefed during the day, bringing the total to 74,091 people sensitized and 6,549 staff and service providers who signed the code of conduct.

2.3.11. Challenges

ÿ Coverage of affected areas remains low: only 56 out of 151 health zones (37%) carry out PSEA activities and only 2 out of 16 partners (12.5%) report their activities; in Ituri, only 15 out of 36 health zones (41.7%) know the reporting mechanism and only one partner (Save the Children) reports its activities; in North Kivu, only 3 out of 34 health zones (8.8%) know the reporting mechanism; in Haut-Uélé, only 6 out of 13 health zones (46.2%) know the reporting mechanism, no partner reports its activities and awareness-raising tools are only available in Swahili, a language spoken in only 3 health zones.

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KEY MESSAGES

Over the past 24 hours, 375 out of 424 suspected cases (88.4%) have been investigated in the four provinces that submitted their data; 49 remain uninvestigated (34 in North Kivu and 15 in Ituri). The contact tracing rate has declined to 87.1% (32,697/37,552), compared to 88.4% the previous day, and remains below the required threshold of 95.0%, with a critical situation in Kayna (0/330) and Manguredjipa (0/71), where no contacts were identified, as well as in Komanda (67.1%), Nia-Nia (70.0%), Beni (71.4%), and Wamba (74.6%). This situation necessitates strengthening the investigation of suspected cases and contact tracing to limit the risk of community transmission.

Maintain a level of vigilance regarding epidemiological surveillance aspects, and consolidate.

TIMELINE OF KEY EVENTS

o From April 24 to May 15, 2026, the epidemic progressed from the detection of the first suspected cases to the biological confirmation of the Ebola Bundibugyo virus, leading to the official declaration of

the 17th Ebola Virus Disease epidemic in the Democratic Republic of Congo.

o Between May 17 and 18, 2026, the national authorities, with the support of partners, strengthened the response by declaring a public health emergency of national and then continental scope, accompanied by the mobilization of funds.

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